INDIAN PRESCRIPTION SHORTHAND & ABBREVIATIONS

Quick reference for the Voice Prescription Writer project

Assignment-focused terminology • Spoken variants • Safety flags • Implementation notes

Purpose
This guide brings together the shorthand explicitly required in the assignment and additional commonly encountered prescription terms that may help with development and testing.

Important: This is a software-development reference, not prescribing guidance. Abbreviations vary by institution and context. Have a qualified clinician approve any mapping before it is used in the application. When a term is ambiguous, preserve the original words and ask the doctor to clarify.

How to read this guide
SECTION A = terms explicitly named in the assignment. SECTION B onward = supplementary reference terms. Supplementary entries must not be treated as required features or universally safe expansions.

A. Shorthand explicitly required by the assignment

These are the terms your application must recognize and normalize, according to the assignment brief.

Shorthand	Meaning in the assignment	Category	Implementation note
OD	Once daily	Frequency	Context-check carefully; OD can also mean right eye in ophthalmology. Prefer writing “daily” in the final output.
BD	Twice daily	Frequency	Preserve the doctor's original wording and confirm if ambiguous.
TDS	Three times daily	Frequency	Do not assume exact clock times unless dictated.
QID	Four times daily	Frequency	Do not convert into fixed times without an instruction.
HS	At bedtime / before sleep	Timing	Preserve whether the doctor said bedtime or a specific time.
SOS	When required / as required	Conditional instruction	Keep the conditional nature; do not turn it into a fixed schedule.
STAT	Immediately / now	Urgency	The assignment includes this as an instruction; retain the one-off/immediate meaning.
1-0-1	Morning - afternoon - night pattern	Dose pattern	Keep the exact three-slot pattern. Do not infer meal timing or exact hours.

Suggested spoken variants for test coverage

Spoken phrase	Possible normalized form	Caution
“Once a day” / “once daily”	OD / daily	Only normalize when the context clearly indicates frequency.
“Twice a day” / “two times daily”	BD	Do not confuse daily frequency with every-12-hour interval unless specifically stated.
“Three times a day”	TDS	Do not assume exact timings.
“Four times a day”	QID	Do not infer clock times.
“At bedtime” / “before sleep”	HS	Keep the phrase if there is any uncertainty.
“When required” / “if needed”	SOS	Preserve any condition or maximum amount if dictated.
“Immediately” / “right now”	STAT	Do not infer repeat dosing.

Note: These spoken variants are examples to help create your own test set. They are not a substitute for clinician-approved terminology.

B. Additional frequency and food-timing terms

These are supplementary terms found in Indian health terminology and clinical reference material. Add them only if your team decides to support them and a clinician validates the intended meanings.

Term	Common meaning	Category	Caution
BID	Twice daily	Frequency	Common synonym of BD in some settings; do not assume every source uses the same notation.
TID	Three times daily	Frequency	Common synonym of TDS.
QDS	Four times daily	Frequency	Common synonym of QID in some settings.
AC	Before meals	Food timing	Do not assume a specific number of minutes before food.
PC	After meals	Food timing	Do not assume a specific number of minutes after food.
A/F	After food	Food timing	Supplementary shorthand; prefer full phrase in final prescription.
B/F	Before food	Food timing	Supplementary shorthand; prefer full phrase in final prescription.
PRN	As needed / when required	Conditional instruction	Do not treat as a fixed frequency; preserve any dictated indication or limits.

C. Interval-based instructions

Term	Common meaning	Important distinction
QH	Every hour	An interval instruction, not simply “once daily” or a daily count.
Q2H	Every 2 hours	Retain the exact interval.
Q4H	Every 4 hours	Do not replace with QID; interval and daily-frequency instructions are not always equivalent.
Q6H	Every 6 hours	Do not calculate or invent a schedule unless explicitly requested.
Q8H	Every 8 hours	Keep the exact interval phrase.

The assignment does not require these interval codes. They are included as supplementary reference only. Unfamiliar interval instructions should be displayed verbatim and sent for doctor review.

D. Numeric dose patterns

Pattern	Illustrative slot reading	Software rule
1-0-1	Morning - afternoon - night	Explicitly required by the assignment.
1-1-1	One in each of the three slots	Only parse if the clinician confirms the clinic's convention.
0-1-0	Middle slot only	Do not infer a meal, clock time, or duration.
1-0-0	First slot only	Do not infer what “first slot” means without a defined convention.

For this project, implement 1-0-1 first because it is explicitly in the assignment. Other patterns are optional and require a doctor-approved convention.

E. Dosage forms and prescription terms

Abbreviation	Meaning	Use in the application
Tab	Tablet	Medicine form
Cap	Capsule	Medicine form
Syp / Syr	Syrup	Medicine form
Susp	Suspension	Medicine form
Inj	Injection	Medicine form; route must be captured separately.
Oint	Ointment	Medicine form
Cr / Cream	Cream	Medicine form
Gtt	Drops	Specify eye/ear/nasal/oral context in full; do not infer.
Amp	Ampoule	Packaging/container term; not itself a route.
Liq	Liquid	General dosage form
Pulv	Powder	General dosage form

F. Route of administration abbreviations

Abbreviation	Meaning	Safety note
PO	By mouth / oral	Prefer “oral” in the final prescription.
IV	Intravenous / into a vein	Preserve exact route; never infer from “injection” alone.
IM	Intramuscular / into a muscle	Preserve exact route.
SC / SQ	Subcutaneous / under the skin	Can be confused with SL. Prefer “subcutaneous” or the locally approved form.
SL	Sublingual / under the tongue	Do not confuse with SC.
PR	Per rectum / rectal	Use full wording in final output.
PV	Per vagina / vaginal	Use full wording in final output.
IN	Intranasal	ISMP identifies IN as potentially confused with IM or IV; write “intranasal”.

Do not infer a route from the dosage form. For example, “injection” does not tell you whether the doctor specified IV or IM.

G. Units and measurements

Notation	Meaning	Preferred display / handling
mg	Milligram	Keep the numeric value and unit together.
mcg	Microgram	Use mcg rather than the micro symbol µg.
g	Gram	Preserve exact number and unit.
mL	Millilitre	Use capital L; avoid “ml” or “cc”.
L	Litre	Use uppercase L.
unit(s)	Units / international units when specified	Write “unit(s)” in full; avoid U or IU.

H. High-risk or ambiguous abbreviations

Medication abbreviations can be misread. The Institute for Safe Medication Practices (ISMP) publishes an error-prone abbreviation list and recommends writing several terms in full. These warnings are especially important for your transcription and normalization logic.

Abbreviation / notation	Risk	Safer handling
OD	Can mean once daily or right eye, depending on context.	Use “daily” for unambiguous frequency; use “right eye” only in a clear ophthalmic context. If ambiguous, ask.
QD / qd	Can be confused with QID.	Avoid; write “daily”.
QOD / qod	Can be confused with daily or four times daily.	Avoid; write “every other day”.
U / u	Can be mistaken for 0, 4, or cc, potentially causing serious dose errors.	Write “unit(s)” in full.
IU	Can be mistaken for IV or the number 10.	Write “unit(s)” in full where appropriate; do not auto-expand.
cc	Can be mistaken for units.	Use mL.
ml	Can be confused in handwritten notation.	Use mL.
µg	Can be mistaken for mg.	Use mcg.
SC / SQ	Can be confused with SL or other notation.	Use “subcutaneous” or the locally approved unambiguous term.
IN	Can be confused with IM or IV.	Write “intranasal”.
Drug-name abbreviations	Different medicines can be confused.	Use the complete medicine name; do not invent expansions.

I. Rules for your Voice Prescription Writer

Rule	What the application should do
Preserve source	Keep the raw audio and verbatim transcript unchanged.
Normalize cautiously	Use only clinician-approved mappings; retain the spoken phrase alongside the normalized value.
No guessing	If a field is absent, keep it empty. If unclear or contradictory, flag it and ask the doctor.
Context matters	Do not expand a token such as OD without checking the surrounding context.
Numbers are exact	Never change 650 to 500 or five days to ten days, or infer a missing value.
Field traceability	Every extracted field should retain its source phrase or audio time range.
Doctor confirmation	Keep final PDF export disabled until the doctor explicitly confirms.
Feedback loop	Store original AI value and doctor-corrected value separately; use reviewed corrections for regression tests and controlled improvements.

J. Recommended fields for your shorthand dictionary

For each mapping, store: token, expanded meaning, category (frequency / timing / route / form / unit), spoken variants, context restrictions, source/reference, clinician approval status, and whether clarification is required when ambiguous.

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Example internal record:
{"token":"BD","meaning":"twice daily","category":"frequency",
"source":"assignment + clinician-checked reference",
"clinician_approved":false,"needs_context_check":true}
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K. Implementation checklist

Priority	Task	Acceptance check
Must-have	Implement assignment terms: OD, BD, TDS, QID, HS, SOS, STAT, 1-0-1.	Pass all relevant supplied dictation scripts.
Must-have	Represent medicine name, strength, form, dose, frequency, timing, duration and route separately.	Each medicine is a separate editable JSON object.
Must-have	Store original audio and raw transcript.	Doctor can review transcript and replay audio.
Must-have	Show field uncertainty and traceability.	Clicking a field reveals its source phrase.
Must-have	Require doctor confirmation before PDF export.	Unconfirmed draft cannot generate final PDF.
Recommended	Use CDCI / approved formulary for medicine-name candidate matching.	Suggestions are never silently substituted.
Recommended	Add reviewed doctor corrections to regression tests.	Previously corrected test cases stay correct after changes.
Optional	Add Hindi/Kannada spoken variants.	Only clinician-approved phrases are normalized.

L. Sources and scope

The assignment-required list on page 2 follows the Voice Prescription Writer brief. Supplementary meanings were cross-checked against the Government of India's Health Domain Metadata and Data Standards medication-frequency code directory and Indian healthcare training references. Safety warnings are drawn from ISMP's 2024 error-prone abbreviations list.

1. Voice Prescription Writer - Intern Assignment
Internal project brief supplied for this assignment (3 November 2026).

2. Ministry of Health and Family Welfare - Health Domain Metadata and Data Standards, Part III: Code Directories
Open government reference. Includes OD, BD, TID, QID, AC, PC, SOS, QH, Q2H, Q4H, HS and STAT.

3. Institute for Safe Medication Practices (ISMP) - List of Error-Prone Abbreviations, Symbols, and Dose Designations (2024)
Open ISMP safety reference. Used for the high-risk abbreviation warnings.

4. National Health Mission, Himachal Pradesh - Palliative Care Training Manual for Staff Nurses
Open training reference. Used only as a supplementary glossary for dosage forms, routes and common clinical notation.

Final reminder: This document is a software-development glossary, not a clinical reference for choosing or changing medication instructions. The application must preserve what the doctor said, surface uncertainty, and leave prescribing decisions to the doctor.